Disorder	What's Happening	Key Clues	Priority Care	NCLEX Trap
Iron Deficiency Anemia	Not enough iron to make hemoglobin. RBCs are usually microcytic and hypochromic.	Fatigue, pallor, cold sensitivity, palpitations, dizziness, brittle hair/nails, cheilosis.	Assess for blood loss, monitor Hgb/Hct, give iron correctly, teach iron-rich foods.	Milk blocks iron absorption. Vitamin C increases absorption. Black stools are expected.
Pernicious Anemia	Vitamin B12 deficiency caused by lack of intrinsic factor. Megaloblastic anemia.	Beefy red tongue, sore mouth, paresthesia, unsteady gait, depression, psychosis.	Give IM B12 injections, monitor neuro status, fall precautions, lifelong teaching.	Oral B12 is wrong if caused by lack of intrinsic factor. Needs lifelong injections.
Folic Acid Deficiency	Folate deficiency affects DNA/RBC production. Megaloblastic anemia.	Fatigue, pallor, weakness, possible glossitis, no neurological symptoms.	Give folic acid, nutrition teaching, review meds, monitor CBC.	Unlike B12 deficiency, folic acid deficiency does not cause neurological symptoms.
Aplastic Anemia	Bone marrow failure causing pancytopenia: low RBCs, WBCs, and platelets.	Fatigue, fever/infection, petechiae, bruising, bleeding.	Neutropenic precautions, bleeding precautions, monitor CBC, assess for infection/bleeding.	If RBCs, WBCs, and platelets are all low, think aplastic anemia. Priority is infection prevention.
Sickle Cell Anemia	Hgb S causes RBCs to sickle during hypoxia, leading to vaso-occlusion and hemolysis.	Severe pain, jaundice, dactylitis, dyspnea, decreased pulses, abdominal pain.	IV fluids first, oxygen, opioids for pain, infection prevention, avoid triggers.	Crisis priority is hydration. No aspirin. No iron. Fever is urgent.
G6PD Deficiency	X-linked enzyme deficiency causing RBC hemolysis after oxidative stress.	Pallor, fatigue, jaundice after trigger exposure.	Avoid trigger meds/foods, medication safety teaching, medic-alert bracelet.	Avoid aspirin, sulfonamides, antimalarials, and fava beans. Males most affected.
Thalassemia	Inherited disorder where body cannot make enough hemoglobin. RBCs are microcytic and hypochromic.	Failure to thrive, delayed growth, splenomegaly, bronzed skin.	Monitor oxygenation, transfusions as needed, monitor iron overload, chelation therapy.	Repeated transfusions can cause iron overload and organ damage.
Autoimmune Hemolytic Anemia	Antibodies attack and destroy the body's own RBCs.	Hemolytic anemia symptoms with cold or warm trigger pattern.	Monitor anemia severity, prevent cold exposure if cold type, treat underlying cause.	Cold type is IgM. Warm type is IgG.
Blood Loss Anemia	RBC loss exceeds production due to acute or chronic bleeding.	Tachycardia, hypotension, pale cool clammy skin, low Hgb/Hct, low O2 sat.	Control bleeding, IV fluids, monitor vitals, prepare for transfusion if ordered.	IV fluids restore volume but dilute the remaining blood. Hgb/Hct may drop after fluids.